

The Complete SEP Training Guide

For Licensed Medicare Advantage Sales Agents

The master training document for Special Enrollment Periods. Every SEP code you will encounter on inbound Medicare calls, organized by category, with talk tracks, real-world scenarios, common mistakes, and side-by-side comparisons.

Audience: Licensed Medicare Advantage sales agents handling inbound calls

Contains: All 37 SEP codes with complete details

Current Plan Year • March 2026

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How to Think About SEPs

What SEPs Are and Why They Exist

Medicare is not like shopping for car insurance. You cannot change your Medicare plan whenever you feel like it. The government controls when beneficiaries can make changes, and outside of specific windows, you cannot enroll anyone in anything. That is the entire reason Special Enrollment Periods exist.

A Special Enrollment Period is a window of time — triggered by a specific life event — during which a Medicare beneficiary is allowed to enroll in, switch, or drop a Medicare Advantage or Part D plan. Without a valid SEP, the enrollment will be rejected. Period.

Think of it this way: Medicare gives every beneficiary a locked door. AEP opens the door once a year for everyone. SEPs are the keys that open that door at other times — but only if the right life event happened, only for the right type of plan change, and only within the right timeframe. Your job is to figure out which key the beneficiary is holding, often before they even realize they have one.

AEP vs OEP vs SEP Season — The Annual Rhythm

Annual Election Period (AEP): Oct 15 – Dec 7

Every beneficiary can enroll, switch, or drop. No qualifying event needed. Coverage effective January 1st. Last plan submitted wins.

MA Open Enrollment Period (OEP): Jan 1 – Mar 31

Only for existing MA members. One change only. Effective 1st of following month. If on Original Medicare with PDP only, OEP does not apply.

SEP Season: Apr 1 – Oct 14

The only way to enroll someone is with a valid SEP. No SEP, no enrollment. This is where knowing these codes separates the professionals from the amateurs.

Critical insight:

Even during AEP and OEP, SEPs still matter. A beneficiary calling in January who has Medicaid does not need to use their one OEP change — they have an INT or DEP SEP that lets them change every single month. Knowledge of SEPs is not just for SEP season. It is for every season.

Standard Enrollment Periods — The Foundation

Before you can understand SEPs, you need to understand the enrollment periods that every beneficiary has access to by default.

PERIOD	WHEN	WHO	WHAT THEY CAN DO	EFFECTIVE DATE
Initial Enrollment Period (IEP)	7 months around 65th birthday	Newly eligible	Enroll in MAPD or PDP	1st of month of eligibility or following month
General Enrollment Period (GEP)	Jan 1 – Mar 31	Missed IEP	Sign up for Part B; enroll in MA/MAPD/PDP	1st of month after sign-up
Annual Election Period (AEP)	Oct 15 – Dec 7	All beneficiaries	Join, switch, or drop MA/MAPD/PDP	January 1
MA Open Enrollment Period (OEP)	Jan 1 – Mar 31	Existing MA/MAPD	One plan change	1st of following month
New-to-MA OEP (OEP-N)	Month of effectuation + 2 months	First-time MA/MAPD	One change if dissatisfied	1st of following month
Medigap OEP	6 months after enrolling in Part B (65+)	Part B enrollees at 65+	Guaranteed issue Medigap	Varies

Why this matters for SEP knowledge:

Every SEP exists because something happened outside these standard windows. Your first job on every call is to determine whether a standard window is open before hunting for an SEP.

CMS Election Period Hierarchy:

When a beneficiary qualifies for more than one enrollment period: (1) IEP/ICEP, (2) MA OEP, (3) SEP, (4) AEP, (5) OEP-Institutional. Always use the one most beneficial to the beneficiary.

Discovery Mindset

The beneficiary almost never calls and says, “I have a Special Enrollment Period.” They call and say, “I just moved to Florida.” They call and say, “My husband passed away and I was on his insurance.” Every one of those statements is a live SEP. Your job is to translate their life into the correct enrollment code.

The best agents on inbound calls are not the ones who know the most about plan benefits. They are the ones who catch SEPs that other agents miss. Every missed SEP is a lost enrollment.

The Most Important Question

“When exactly did that happen?”

SEPs are time-sensitive. The difference between an enrollment and a dead call is often a single date. Get the date. Get it early. Get it exact. Then calculate the window, state the deadline, and create urgency.

“Mrs. Johnson, based on that date, you have 47 days left in your enrollment window. Let’s make sure we get you into the right plan before that closes.”

Time-Sensitive vs Always-Available SEPs

Time-sensitive SEPs have windows that open and close on specific dates. Once closed, they are gone. Examples: MOV, LEC, IEP. When identified, urgency is your friend.

Always-available SEPs do not expire as long as the qualifying condition persists. Examples: INT, DEP, CDC, OEP-I. The reframe: “you have options right now” instead of “you are stuck.”

The SEP Categories

Chapter 1: New to Medicare

Where every Medicare journey begins. Five codes in this category, and confusing them is one of the most common mistakes agents make.

IEP

Initial Enrollment Period

The beneficiary is turning 65 and getting Medicare for the first time. This is their original, default enrollment window. Seven months total.

WHAT TRIGGERS IT

Turning 65 years old, or becoming eligible for Medicare due to disability after 24 months on SSDI. Part A and Part B must share the same effective date.

ENROLLMENT WINDOW

Seven months — three months before the 65th birthday month, the birthday month itself, and three months after.

WHO QUALIFIES

Anyone turning 65 with both Part A and Part B starting on the same date. Also applies to disability beneficiaries reaching 24 months of SSDI. NOT already in an active MA/MAPD plan. NOT valid for MA-only plans — must include drug coverage (MAPD or standalone PDP).

COMMON MISTAKES

Confusing IEP with ICEP. The single distinguishing question: do Part A and Part B have the same effective date? If yes, IEP. If different, ICEP. Trying to use IEP after a plan has already effectuated — once in an active plan, IEP is no longer available (use OEP-N).

TALK TRACK

“Since you are right in your initial enrollment window, we can get you set up with the right plan today — no special circumstances needed. You have a seven-month window, and I want to make sure we use it well.”

REAL-WORLD SCENARIO

Mr. Williams mentions he just turned 65 last month. MARx shows Part A and Part B share the same effective date. No existing plan on file. He is in his IEP with five months remaining. You tell him exactly when his window closes and move to plan comparison.

IEP2

Disability Beneficiary Turning 65

Someone who has been on Medicare through disability is now turning 65, and they get a brand-new seven-month window — as if they were enrolling for the very first time.

WHAT TRIGGERS IT

A Medicare beneficiary who originally qualified through disability (under 65) is now turning 65. MARx will show a Medicare Reason for Disability (MRD) code. Calculate the current golden birth year by subtracting 65 from the current year.

ENROLLMENT WINDOW

Seven months — identical structure to IEP. Three months before the 65th birthday month, the birthday month, and three months after. Uses MRD on the application.

WHO QUALIFIES

Disability beneficiaries turning 65. They already have Medicare through disability, but they get a completely fresh enrollment window at 65. MA-only plans are prohibited — must enroll in MAPD or PDP.

COMMON MISTAKES

Agents do not realize this is a separate, fresh window. The beneficiary may already be in a plan through their disability enrollment, but IEP2 gives them a new opportunity. Do not confuse their existing disability-era enrollment with using their IEP2. MA-only plans are prohibited.

TALK TRACK

“Because you are turning 65 and you have been on Medicare through disability, you actually get a brand-new enrollment window — a fresh start. We can look at all the plans available to you right now.”

REAL-WORLD SCENARIO

Mrs. Thompson has been on Medicare since age 58 due to disability. She is turning 65 next month. MARx shows the MRD indicator. She has a fresh IEP2 window opening up — seven months to get her into the right MAPD plan.

ICEP

Initial Coverage Election Period

The beneficiary delayed Part B — usually because they had employer coverage — and is now activating it. Part A and Part B have different effective dates.

WHAT TRIGGERS IT

Part B activation after a delay. Part A and Part B have different effective dates in MARx. Most often when someone had employer coverage past 65, kept Part A, delayed Part B, and is now activating Part B because they retired or lost employer coverage.

ENROLLMENT WINDOW

Five months — three months before the Part B effective month, the Part B effective month, and one month after. Recently extended from the previous shorter duration.

WHO QUALIFIES

Beneficiaries whose Part A and Part B have different effective dates. NOT valid for PDP enrollment — ICEP is for MA or MAPD only. If they need a standalone PDP, use IEP instead.

COMMON MISTAKES

Using ICEP when Part A and Part B share the same date (that is IEP). Using ICEP for PDP enrollment (not valid). Using outdated window calculations. ICEP often coincides with LEC — if employer coverage is ending at the same time Part B activates, document both codes.

TALK TRACK

“Since your Part B is just now starting, you are in your initial enrollment window for a Medicare Advantage plan. We have a five-month window to work with — let me show you what is available in your area.”

REAL-WORLD SCENARIO

Mr. Garcia is 68. He had Part A since turning 65 but kept his employer insurance and delayed Part B. He just retired, Part B starts next month. MARx shows different effective dates. This is ICEP, not IEP. You also check for LEC since employer coverage is ending.

OEP-N

New Enrollee OEP

The beneficiary just enrolled in their very first MA or MAPD plan, it has gone into effect, and they do not like it. They get one chance to change.

WHAT TRIGGERS IT

First-time MA/MAPD enrollment that has effectuated. The beneficiary realizes the plan is not right — network does not include their doctor, formulary misses their medications.

ENROLLMENT WINDOW

Month of effectuation plus two months. If the plan went into effect January 1st, they have January, February, and March.

WHO QUALIFIES

First-time MA/MAPD enrollees only. The plan must have actually effectuated — a pending enrollment does not trigger OEP-N. Cannot be used by PDP-only or Medigap beneficiaries. One change only — once used, it is gone.

COMMON MISTAKES

Trying to use OEP-N for someone with prior MA history — it is for first-timers only. Trying to use it before the plan has effectuated. Trying to use it for a PDP-only beneficiary. Not realizing that once OEP-N is used, it is consumed forever.

TALK TRACK

“Since this is your first Medicare Advantage plan and it just went into effect, you actually have a window right now to make one change if it is not the right fit. Let’s look at what else is available.”

REAL-WORLD SCENARIO

Mrs. Davis enrolled in an MAPD plan during AEP. It went into effect January 1st. She calls in February because her PCP is not in the network. She has never been in an MA plan before. OEP-N — she has until the end of March. One shot.

RET

Retroactive Entitlement

The beneficiary was retroactively enrolled in Medicare — their Part A and/or Part B started without them knowing, and they only found out after the fact.

WHAT TRIGGERS IT

Notification of Medicare entitlement after coverage has already begun. Commonly surfaces when Social Security retroactively awards disability or Medicare benefits. Clock starts from notification date, not coverage date.

ENROLLMENT WINDOW

Month of notification plus two full calendar months.

WHO QUALIFIES

Beneficiaries notified of Part A and/or Part B after their coverage had already started. If they have both Part A and Part B, both effective dates must be the same (if different, use ICEP instead).

COMMON MISTAKES

Calculating the window from the coverage effective date instead of the notification date. Confusing RET with ICEP when Part A and Part B have different dates.

TALK TRACK

“It sounds like you were enrolled in Medicare before you even knew about it. The good news is that gives you a window right now to choose a plan. Let me walk you through what is available.”

REAL-WORLD SCENARIO

Mr. Robinson calls confused. He received a letter from Social Security last week saying his Part A and Part B have been active since January. He had no idea. His notification date is this month, so his RET window is this month plus two more.

Comparison: IEP vs ICEP

	IEP	ICEP
The one question	Do Part A and Part B share the same effective date?	Are the dates different?
If YES	Use IEP	Use ICEP
Window	7 months around 65th birthday	5 months around Part B start
Anchor date	65th birthday	Part B effective date
PDP eligible?	Yes	No — ICEP is MA/MAPD only
MA-only eligible?	No	Yes
Common overlap	None typical	Often overlaps with LEC

Chapter 2: Financial Eligibility

SEPs tied to Medicaid and Extra Help (LIS). If you work inbound Medicare calls, you will use these codes more than almost any others.

INT

Integrated Care SEP

The beneficiary has full Medicaid and wants to enroll in a D-SNP (Dual-Eligible Special Needs Plan). They can do this any month, as many times as they want.

WHAT TRIGGERS IT

Full Medicaid status. The beneficiary must have FBDE (Full Benefit Dual Eligible), QMB+, SLMB+, or Full Medicaid. Partial levels like QMB-only do NOT qualify for INT.

ENROLLMENT WINDOW

Any month. Repeatable. No annual limit.

WHO QUALIFIES

Beneficiaries with full Medicaid enrolling into a D-SNP only. Verify Medicaid level before using this code. QMB-only does NOT qualify — they need QMB+ or higher. The D-SNP must be an integrated plan: FIDE SNP, HIDE SNP, or AIP. The Medicaid MCO must match the carrier offering the D-SNP.

COMMON MISTAKES

Using INT for someone with QMB-only (not eligible). Using INT for a PDP enrollment (INT is D-SNP only — use DEP for PDP). Not verifying Medicaid level before submitting. Enrolling into a D-SNP that is not FIDE, HIDE, or AIP.

TALK TRACK

“Since you have full Medicaid, you have the ability to enroll in a Special Needs Plan designed specifically for people with both Medicare and Medicaid. These plans coordinate your benefits and often have zero or very low out-of-pocket costs. And you can make this change any time — you are not locked in.”

REAL-WORLD SCENARIO

Mrs. Hernandez mentions the state pays for everything. You confirm full Medicaid (FBDE). She is in a standard MAPD plan. An aligned integrated D-SNP is available in her county through Humana. You enroll her using INT. If next month she wants to switch to a different D-SNP, she can.

MCO Matching — Critical Detail: INT can only be used to enroll into one of three integrated D-SNP types: FIDE SNP, HIDE SNP, or D-SNP that is an AIP. The MCO on the Medicaid side must match the carrier offering the D-SNP. Check carrier portals and state Medicaid managed care directories to confirm alignment.

DEP

Dual/LIS Monthly SEP

The beneficiary has any level of Medicaid or Extra Help/LIS and wants to change their standalone Part D drug plan. Any month. Repeatable.

WHAT TRIGGERS IT

Any level of Medicaid — including QMB-only, SLMB-only, QI, and all higher levels. Or any level of Extra Help/LIS — Auto, Full, or Partial tiers.

ENROLLMENT WINDOW

Any month. Repeatable every month. No limit.

WHO QUALIFIES

Anyone with any Medicaid level or any LIS/Extra Help level. DEP is for PDP enrollment only. Cannot use DEP for MA or MAPD plans. If they want a D-SNP, use INT instead (and verify full Medicaid).

COMMON MISTAKES

Trying to use DEP for MA or MAPD enrollment (PDP only). Confusing DEP with INT — DEP has a lower bar (any Medicaid/LIS) but limited to PDP. INT has a higher bar (full Medicaid) but allows D-SNP. Confusing the ongoing monthly DEP with the one-time NLS change-in-status SEP.

TALK TRACK

“Since you have Medicaid [or Extra Help], you actually have the ability to change your drug plan every single month — you are not locked in. If we find something better today, we can get that started for the first of next month.”

REAL-WORLD SCENARIO

Mr. Chen has QMB-only — the state pays his Part B premium, but he does not have full Medicaid. He is unhappy with his current PDP’s formulary. QMB-only does not qualify for INT, but it qualifies for DEP. You can switch his PDP today.

The LIS Two-SEP Distinction: LIS beneficiaries have access to two different SEPs. (1) Ongoing monthly DEP — available every month, repeatable, for switching between standalone PDPs. (2) One-time change-in-status SEP (NLS) — triggered when LIS level actually changes, gives a 3-month window and allows MAPD or PDP enrollment. If a beneficiary’s LIS status just changed, they have both DEP and NLS — use NLS for broader plan options.

MCD

Medicaid Change SEP

The beneficiary’s Medicaid status recently changed — they gained it, lost it, or their level shifted. Three-month window.

WHAT TRIGGERS IT

Any change in Medicaid level. Gained Medicaid for the first time. Lost Medicaid entirely. Shifted from QMB-only to full Medicaid. Any change counts.

ENROLLMENT WINDOW

Three months from the date the Medicaid change took effect. Available all year — including after September 30th.

WHO QUALIFIES

Any beneficiary whose Medicaid level changed within the last three months.

COMMON MISTAKES

Calculating the window from the call date instead of the Medicaid change date. Not realizing MCD is available past September 30th. Missing the trigger because the beneficiary does not explicitly say "my Medicaid changed" — listen for green, purple, or orange Social Security letters.

TALK TRACK

"Since your Medicaid situation just changed, you have a three-month window to adjust your Medicare plan. Let's make sure you are in the right plan for your current situation."

REAL-WORLD SCENARIO

Mrs. Patterson calls in October. It is after September 30th, so most agents would think no enrollment is possible until AEP. But she just got approved for full Medicaid last month. That Medicaid change triggers MCD — a three-month window, available all year. She can enroll in a D-SNP right now using both MCD and INT.

NLS

Extra Help Change SEP

The beneficiary's Extra Help (LIS) level recently changed — gained, lost, or shifted between tiers. Three-month window.

WHAT TRIGGERS IT

Any change in Extra Help/LIS status. Gained LIS for the first time. Lost LIS entirely. Shifted between tiers (Auto to Full, Full to Partial, etc.).

ENROLLMENT WINDOW

Three months from the date of the LIS change. Available all year — including after September 30th.

WHO QUALIFIES

Any beneficiary whose Extra Help/LIS level changed within the last three months.

COMMON MISTAKES

Not listening for the trigger. Beneficiaries rarely say "my Extra Help level changed." They say things like "I got a letter from Social Security" or "my copays went up." Green, purple, or orange letters from Social Security signal an Extra Help change. If Medicaid changed at the same time, MCD applies too.

TALK TRACK

"It sounds like your Extra Help benefits recently changed. That gives you a special window right now to switch to a plan that better fits your current situation."

REAL-WORLD SCENARIO

Mr. Adams calls in November. He got a purple letter from Social Security saying his Extra Help level changed from Full to Partial. His copays on medications just went up. NLS window is three months from the change date — and since NLS is available past September 30th, you can help him today.

WARNING: GIVE-BACK PLANS AND MEDICAID BENEFICIARIES

Never enroll a Medicaid beneficiary in a “give-back” plan. Medicaid already pays the Part B premium. A give-back plan would reduce a premium the beneficiary is not paying — the savings go nowhere. Steer toward D-SNPs, not give-back plans.

Comparison: INT vs DEP

	INT	DEP
The one question	Does the beneficiary have FULL Medicaid?	Do they have ANY Medicaid or LIS?
Medicaid level required	Full: FBDE, QMB+, SLMB+, Full	Any: QMB-only, SLMB-only, QI, LIS-only — all qualify
What can they enroll in?	D-SNP only	PDP only
Repeatable?	Yes, any month	Yes, any month
Key trap	QMB-only does NOT qualify	Cannot use for MA/MAPD

Chapter 3: Location and Life Changes

SEPs triggered by physical changes in the beneficiary's life — where they live, where they came from, their legal status.

MOV

Change of Permanent Residence SEP

The beneficiary moved to a new address where their current plan may not be available. They have a window to enroll in a plan that serves their new location.

WHAT TRIGGERS IT

A change of physical residence to a different ZIP code or county. House fire, eviction, or moving in with family all count as long as it is a different physical address. System address mismatch is a potential live SEP.

ENROLLMENT WINDOW

Month before the move (if notified in advance) plus the month of the move plus two full calendar months after the move.

WHO QUALIFIES

Any beneficiary who moved to a new physical address. Can enroll in any MA, MAPD, or PDP available at the new address.

COMMON MISTAKES

Accepting a PO Box change as a MOV trigger — it is NOT valid. Must be a physical address change. Updating the address in the system and moving on without offering enrollment. Missing the SEP because the agent does not ask about the address or check for mismatches.

TALK TRACK

“Because you have moved to a new area, you are actually in a special enrollment window right now. We can get you set up with the right plan for your new address today. Based on your move date, you have [X] days left in that window.”

REAL-WORLD SCENARIO

Mrs. Rodriguez calls about her plan. You verify her address and she gives a Florida address — but MARx shows Texas. She moved three weeks ago. That address mismatch is a live MOV SEP. You confirm the move date, calculate the window, and move to plan comparison for her new Florida address.

INC

Post-Incarceration SEP

The beneficiary was recently released from a correctional facility and needs to re-enroll in a Medicare plan.

WHAT TRIGGERS IT

Release from a jail, prison, or detention center. Medicare is suspended — not terminated — during incarceration, so Part A and Part B should still be intact upon release.

ENROLLMENT WINDOW

Two months from the release date.

WHO QUALIFIES

Any Medicare beneficiary released from incarceration within the last two months. Can enroll in MA, MAPD, or PDP.

COMMON MISTAKES

Assuming Medicare was terminated during incarceration — it is suspended, not cancelled. Calculating the window from the call date instead of the release date. Not knowing that pre-release planning is possible — enrollment can be submitted up to one month before the release date.

TALK TRACK

“Welcome back. Since you were just released, you have a two-month window to get enrolled in a Medicare plan. Your Medicare should still be active — let me verify that and then we can look at what is available.”

REAL-WORLD SCENARIO

Mr. Jackson says he just got out of a correctional facility two weeks ago and needs health insurance. Part A and Part B are still active (suspended during incarceration, now reinstated). Release date was two weeks ago, so he has about six weeks left in his INC window.

RUS

Return to US SEP

The beneficiary was living permanently outside the United States and has now returned. They have two months to enroll.

WHAT TRIGGERS IT

Returning to the US after permanent residence abroad. Short trips and vacations do NOT qualify — this is for beneficiaries who were living outside the US full-time.

ENROLLMENT WINDOW

Two months from the return date.

WHO QUALIFIES

Medicare beneficiaries who permanently resided outside the US and have now returned. Can enroll in MA, MAPD, or PDP.

COMMON MISTAKES

Applying RUS to someone who was on vacation or a short trip. Not verifying that Part A and Part B are still active after extended absence. Not confirming the US return address is current in the system.

TALK TRACK

“Welcome back to the States. Since you have been living abroad, you have a special enrollment window to get set up with a Medicare plan here. Let me check your Medicare status and we will find the right plan for your area.”

REAL-WORLD SCENARIO

Mrs. Kim has been living in South Korea with family for three years and just moved back to California last month. You verify Part A and Part B are still active, confirm her new California address, and proceed to enrollment under RUS.

LAW

Lawful Presence SEP

The beneficiary recently became a US citizen or gained lawful status that makes them newly eligible for Medicare.

WHAT TRIGGERS IT

Becoming a US citizen or acquiring qualifying lawful presence that triggers Medicare eligibility for the first time.

ENROLLMENT WINDOW

Month of the status change plus two full calendar months.

WHO QUALIFIES

Beneficiaries who recently crossed the eligibility threshold through citizenship or lawful presence. Can enroll in MA, MAPD, or PDP.

COMMON MISTAKES

Assuming citizenship automatically starts Medicare — it does not. Part A and Part B must be confirmed active or activating. Some non-citizens may have waiting periods for premium-free Part A.

TALK TRACK

“Congratulations on your citizenship. That actually opens a Medicare enrollment window for you right now. Let me check your Medicare eligibility and we will get you set up.”

REAL-WORLD SCENARIO

Mr. Nguyen recently became a US citizen at age 67. He has been told he now qualifies for Medicare. You verify Part A and Part B are being activated, confirm his address, and enroll him using LAW.

Comparison: MOV vs LEC

	MOV	LEC
The one question	Did they physically move?	Did they lose employer/union/COBRA coverage?
Trigger	New physical address	Coverage termination
Window	Month before (if notified) + month of + 2 months after	Month of loss + 2 months
What they enroll in	Any plan at new address	Any MA, MAPD, or PDP
Key trap	PO Box change does NOT count	COBRA expiration counts as loss
Overlap	May overlap with LEC if move caused job loss	May overlap with ICEP if Part B also activating

Chapter 4: Chronic and Special Needs

Some beneficiaries have conditions or circumstances requiring specialized plan types. CSN is the single biggest enrollment opportunity outside of AEP.

CSN

C-SNP Eligibility SEP

The beneficiary has a qualifying chronic condition and a C-SNP (Chronic Condition Special Needs Plan) is available in their county for that condition. They can enroll.

WHAT TRIGGERS IT

The beneficiary has a severe or disabling chronic condition — diabetes, heart failure, ESRD, COPD, cancer, and many others — and a matching C-SNP exists in their county. The condition alone is not enough; a C-SNP for that specific condition must be available where they live.

ENROLLMENT WINDOW

Once per calendar year per qualifying condition. A new qualifying condition triggers a new SEP.

WHO QUALIFIES

Beneficiaries with a qualifying chronic condition enrolling INTO a C-SNP. Must be enrolling into a C-SNP — cannot be used for standard MA or MAPD. C-SNP to C-SNP for the same condition is NOT valid.

COMMON MISTAKES

Confirming the SEP before verifying a C-SNP actually exists in the beneficiary's county. Not all counties have C-SNPs. Check Aetna, Humana, UHC, and WellCare carrier portals first. C-SNP to C-SNP for the same condition is NOT valid — only switching for a different condition qualifies.

TALK TRACK

“Because you have [condition], you may actually qualify for a Special Needs Plan built specifically for people managing that. These plans often have lower copays and benefits tailored to your exact needs — and you can enroll right now.”

REAL-WORLD SCENARIO

Mrs. Washington mentions she takes metformin and insulin for her diabetes. You check Humana's carrier portal and find a Diabetes Management C-SNP in her county. She qualifies for CSN. You explain the tailored benefits and proceed to enrollment.

Qualifying chronic conditions: Chronic alcohol/drug dependence, autoimmune disorders (lupus, rheumatoid arthritis, MS), cancer, cardiovascular disorders, chronic heart failure, dementia/Alzheimer's, diabetes, end-stage liver disease, ESRD/dialysis, hematologic disorders, HIV/AIDS, chronic lung disorders (COPD, emphysema), chronic mental health conditions (schizophrenia, bipolar, major depression), neurological disorders (Parkinson's, epilepsy, ALS), and stroke/CVA.

Provider attestation: C-SNP enrollment requires the beneficiary's provider to attest that the beneficiary has been diagnosed with the qualifying condition. This must be completed prior to the end of the second month of enrollment. If not confirmed, the beneficiary will be disenrolled but gets a two-month SEP (SNP code) to join another plan.

Why C-SNP is your biggest SEP-season tool: 17 million eligible beneficiaries. Year-round availability. No ticking clock. High-value plans with lower copays for condition-specific services. The question that opens the door: "Do you have any ongoing health conditions like diabetes, heart failure, or COPD?"

PAP SPAP SEP

The beneficiary is enrolled in an approved State Pharmaceutical Assistance Program and can change their PDP once per year, or has two months after losing that program.

WHAT TRIGGERS IT

Active enrollment in a CMS-approved SPAP program. Common qualifying programs: New York (EPIC), New Jersey (PAAD), Pennsylvania (PACE/PACENET), Wisconsin (SeniorRx).

ENROLLMENT WINDOW

One PDP change per year while actively enrolled. If they lose the SPAP enrollment, two months from the loss date.

WHO QUALIFIES

Beneficiaries currently enrolled in a CMS-approved SPAP program. PDP enrollment or changes only.

COMMON MISTAKES

Assuming all state assistance programs qualify — only CMS-approved SPAP programs count. Not confirming the beneficiary is still actively enrolled in the program.

TALK TRACK

"Since you are enrolled in [state program], you have the ability to change your drug plan right now. Let's see if there is something that works better with your current medications."

REAL-WORLD SCENARIO

Mr. O'Brien from New York mentions he is in the EPIC program. You confirm EPIC is a CMS-approved SPAP. He has not used his annual PAP change yet this year. You can switch his PDP today.

PAC PACE Disenrollment SEP

The beneficiary has already left a PACE program on their own. They have two months to enroll in a new plan.

WHAT TRIGGERS IT

Voluntary disenrollment from a PACE (Program of All-Inclusive Care for the Elderly) plan — which has already occurred. You should never suggest or initiate PACE disenrollment.

ENROLLMENT WINDOW

Two months from the PACE disenrollment date.

WHO QUALIFIES

Beneficiaries who have already disenrolled from PACE. Can enroll in MA, MAPD, or PDP.

COMMON MISTAKES

Encouraging or initiating PACE disenrollment — NEVER do this. PACE provides comprehensive, coordinated care for frail elderly individuals. This SEP only applies after disenrollment has already happened on the beneficiary's initiative.

TALK TRACK

"Since you have already left the PACE program, you have a two-month window to get enrolled in a new plan. Let me show you what is available."

REAL-WORLD SCENARIO

Mrs. Foster left her PACE program two weeks ago because she moved to a new area where PACE is not available. She has about six weeks left in her PAC window. You confirm the disenrollment date and proceed.

SNP

SNP Loss SEP

The beneficiary is in a Special Needs Plan but has lost their SNP eligibility — either because the qualifying condition was not verified in time or they no longer meet the criteria.

WHAT TRIGGERS IT

Loss of SNP eligibility. Most commonly: the provider failed to verify the qualifying chronic condition within two months of enrollment. Or the beneficiary no longer meets the SNP eligibility criteria.

ENROLLMENT WINDOW

From the time of SNP eligibility loss up to three months after the SNP's grace period ends.

WHO QUALIFIES

Beneficiaries currently in a C-SNP (or other SNP) who have lost eligibility. They need a new plan before the SNP disenrollment date to avoid a coverage gap.

COMMON MISTAKES

Not acting fast enough — the window is measured from the grace period end, not the call date. Not realizing that provider failure to verify is common and often not the beneficiary's fault.

TALK TRACK

"It looks like your Special Needs Plan eligibility has changed. The good news is you have a window right now to get into a new plan so there is no gap in your coverage. Let me help you with that."

REAL-WORLD SCENARIO

Mr. Brown calls confused. He got a letter saying his C-SNP enrollment is being terminated because his doctor never sent in the condition verification paperwork. This is not his fault. You calculate how much of his SNP window remains and move quickly to enroll him in a new plan.

Chapter 5: Institutionalized and Long-Term Care

Two codes split along one simple line: MA/MAPD vs PDP.

OEP-I

Institutionalized OEP

The beneficiary lives in a nursing home, SNF, or LTC facility. They can enroll in or switch MA/MAPD plans at any time — unlimited changes while in the facility, plus two months after discharge.

WHAT TRIGGERS IT

Current residence in a qualifying care facility: nursing home, skilled nursing facility, or long-term care facility.

ENROLLMENT WINDOW

Unlimited while in the facility. Plus two full months after discharge.

WHO QUALIFIES

Beneficiaries currently living in a qualifying facility. Must enroll in MA or MAPD only — NOT PDP. For PDP enrollment in a care facility, use LTC instead.

COMMON MISTAKES

Applying OEP-I to assisted living facilities or residential homes — they do NOT qualify. Must be a licensed nursing home or SNF. Using OEP-I for PDP enrollment (use LTC). Not confirming the beneficiary is still in the facility.

TALK TRACK

“Because you are in a care facility right now, you have the ability to change your Medicare Advantage plan at any time. There is no deadline pressure — we can make sure you are in the plan that serves you best.”

REAL-WORLD SCENARIO

Mrs. Grant’s daughter calls on her behalf. Mrs. Grant is in a skilled nursing facility recovering from a hip replacement. She is unhappy with her MAPD coverage for rehab services. OEP-I applies — unlimited changes. You switch her to an MAPD with better rehab coverage.

LTC

LTC SEP

Same facility requirement as OEP-I, but for standalone PDP enrollment instead of MA/MAPD.

WHAT TRIGGERS IT

Current residence in a qualifying care facility: skilled nursing facility, nursing home, intermediate care facility (mentally disabled), psychiatric hospital, rehabilitation hospital, or long-term care hospital.

ENROLLMENT WINDOW

Unlimited while in the facility. Plus two full months after discharge.

WHO QUALIFIES

Beneficiaries in a qualifying facility who need a standalone PDP. Must enroll in PDP only — NOT MA/MAPD (use OEP-I for that).

COMMON MISTAKES

Confusing LTC with OEP-I. They cover the same facility types, but LTC is PDP-only and OEP-I is MA/MAPD-only. Applying to assisted living facilities (not qualifying).

TALK TRACK

“Since you are in a care facility, you can change your drug plan at any time. Let me look at what is available and find the best one for your current medications.”

REAL-WORLD SCENARIO

Mr. Mitchell is in a long-term care hospital. He is on Original Medicare (not MA) and needs a better Part D plan. LTC applies — he can switch PDPs at any time while in the facility.

Comparison: OEP-I vs LTC

	OEP-I (LT2)	LTC
The one question	Do they need MA/MAPD?	Do they need a PDP?
Enrolls into	MA or MAPD only	Standalone PDP only
Facility types	Same qualifying facilities	Same qualifying facilities
Window	Unlimited + 2 months after discharge	Unlimited + 2 months after discharge
Key trap	Assisted living does NOT qualify	Assisted living does NOT qualify

Chapter 6: Involuntary Disenrollment

The beneficiary loses coverage through no fault of their own — plan leaves the area, Medicare terminates the contract, they lose creditable coverage.

LCC

Loss of Creditable Coverage

The beneficiary lost other creditable health or drug coverage — VA, TRICARE, employer, ACA — and now needs a Medicare plan.

WHAT TRIGGERS IT

Involuntary loss of creditable coverage. VA benefits ending, TRICARE ending, employer coverage ending, ACA marketplace coverage ending. The loss must be involuntary — if the beneficiary stopped paying premiums and lost coverage, LCC does NOT apply.

ENROLLMENT WINDOW

Two months from the date of loss OR the date of notification — whichever is later.

WHO QUALIFIES

Beneficiaries who involuntarily lost creditable coverage. Can enroll in MA, MAPD, or PDP.

COMMON MISTAKES

Using LCC when the beneficiary forfeited coverage by not paying premiums — that is voluntary. Calculating from the loss date when the notification date was later. Not recognizing that VA and TRICARE are creditable coverage.

TALK TRACK

“Since you just lost your [VA/TRICARE/employer] coverage, you qualify for a special enrollment window right now. We need to get you into a plan before that window closes. When exactly did that coverage end?”

REAL-WORLD SCENARIO

Mrs. Clark’s husband was a veteran and they had VA coverage. He passed away last month and she lost her VA health benefits. You confirm the loss date, calculate two months from that date (or from notification, whichever is later), and proceed.

INV

Involuntary Loss SEP

The beneficiary lost their Medicare Advantage plan because their Part B was terminated or dropped. They need a PDP to keep drug coverage.

WHAT TRIGGERS IT

Loss of MA/MAPD plan because Part B was terminated. Without Part B, you cannot be in Medicare Advantage. The beneficiary falls back to Original Medicare (Part A only) and needs a standalone PDP.

ENROLLMENT WINDOW

From notice of Part B loss, through the plan's grace period, plus two months after coverage ends.

WHO QUALIFIES

Beneficiaries who lost MA/MAPD due to Part B termination. Must enroll in PDP only — cannot re-enter MA until Part B is reinstated.

COMMON MISTAKES

Trying to enroll in MA when Part B is not active. Not realizing that when Part B is eventually restored, ICEP or another qualifying SEP will apply for MA re-enrollment.

TALK TRACK

"It looks like your Part B was dropped, which means your Medicare Advantage plan ended too. The important thing right now is to get you into a drug plan so your prescriptions are covered. When Part B gets reinstated, we can look at getting you back into an Advantage plan."

REAL-WORLD SCENARIO

Mr. Taylor calls in a panic. His MAPD plan is ending because his Part B was terminated — he missed a premium payment. You enroll him in a PDP using INV and advise him to contact Social Security to reinstate Part B.

REC

Receivership SEP

The beneficiary's insurance carrier has been taken over by the state due to financial instability. They can switch plans at any time until the situation resolves.

WHAT TRIGGERS IT

State financial receivership of the beneficiary's plan carrier. This is rare — CMS and the state communicate directly with affected beneficiaries.

ENROLLMENT WINDOW

From the effective date of state action until the action ends or the beneficiary enrolls in a new plan — whichever comes first.

WHO QUALIFIES

Beneficiaries whose current carrier is under active state financial receivership. Can switch to any MA, MAPD, or PDP.

COMMON MISTAKES

Confusing REC with EOC — REC is a financial solvency issue, EOC is a carrier market exit. Verify receivership status with your compliance team before using this code.

TALK TRACK

"Your current carrier is going through some financial changes, and CMS is offering you the ability to switch to any other plan right now. Let's find one that maintains the coverage you need."

REAL-WORLD SCENARIO

Mrs. White calls referencing a letter about her insurance company being "taken over by the state." You verify with your compliance team that the carrier is indeed under receivership. REC applies — she can switch to any plan immediately.

EOC

Plan Non-Renewal SEP

The beneficiary's carrier decided to stop offering their plan in their area. The plan is ending, and they need a new one.

WHAT TRIGGERS IT

Carrier non-renewal. The carrier decided to leave the beneficiary's service area — they were not terminated by Medicare, they chose to exit. The beneficiary should have received an ANOC letter.

ENROLLMENT WINDOW

December 8th through the end of February.

WHO QUALIFIES

Beneficiaries whose plan was non-renewed by the carrier. Can enroll in any MA, MAPD, or PDP.

COMMON MISTAKES

Confusing EOC with MYT — EOC is carrier-initiated (they chose to leave), MYT is CMS-initiated (Medicare terminated the contract). Not checking the system for plan termination status.

TALK TRACK

"It looks like your current plan is actually leaving your area — which means you automatically qualify for a special enrollment window to get into a new plan. Let's make sure we get you set up before your current coverage ends."

REAL-WORLD SCENARIO

Mr. Lopez calls in January. His Humana MAPD plan is no longer being offered in his county. EOC applies. He has until the end of February. You also note that since he is in an MA plan during January through March, OEP may also be available.

MYT

Medicare Contract Termination

Medicare itself terminated the contract with the beneficiary's plan carrier. CMS-initiated, not carrier-initiated.

WHAT TRIGGERS IT

CMS has terminated its contract with the plan carrier. This is a CMS enforcement action — different from a carrier voluntarily leaving a market.

ENROLLMENT WINDOW

Two months before the contract end date plus one full month after.

WHO QUALIFIES

Beneficiaries whose plan carrier’s contract was terminated by CMS. Can enroll in any MA, MAPD, or PDP.

COMMON MISTAKES

Confusing MYT with EOC. EOC = carrier chose to leave. MYT = Medicare terminated the contract. Different trigger, different window, different code.

TALK TRACK

“Medicare has ended the contract with your current plan’s carrier, so your coverage will be changing. The good news is you have a window right now to choose a new plan. Let me help you find the right one.”

REAL-WORLD SCENARIO

Mrs. Anderson received a formal CMS letter saying her plan’s contract is being terminated effective June 30th. MYT applies. Her window opens two months before termination (April) and extends one month after (July).

Comparison: LCC vs LEC

	LCC	LEC
The one question	Did they lose creditable coverage that is NOT employer-based?	Did they lose employer/union/COBRA coverage?
Covers	VA, TRICARE, ACA, other creditable coverage	Employer, union, COBRA specifically
Window	2 months from loss or notification (later)	Month of loss + 2 months
Key trap	NOT valid if they stopped paying premiums	COBRA expiration counts
Overlap	May overlap with MOV	May overlap with ICEP

Comparison: EOC vs MYT

	EOC	MYT
The one question	Did the carrier choose to leave?	Did Medicare terminate the contract?
Who initiated?	Carrier decided to exit the market	CMS enforcement action
Window	Dec 8 through end of February	2 months before + 1 month after contract end
Beneficiary notice	ANOC letter from carrier	Formal CMS letter
Key trap	Overlaps with OEP for existing MA members	Enrollment takes effect after contract end date

Chapter 7: Voluntary Changes

Triggered by the beneficiary's own actions — leaving employer coverage, dropping a cost plan, exercising a trial right, or having other drug coverage.

LEC

Loss of Employer Coverage SEP

The beneficiary lost employer, union, or COBRA coverage — through retirement, layoff, COBRA expiration, or death of a covered spouse.

WHAT TRIGGERS IT

Loss of employer, union, or COBRA coverage for any reason. Retirement. Layoff. COBRA expiration. Death of a spouse who carried the employer coverage. All count.

ENROLLMENT WINDOW

Month of loss plus two full calendar months.

WHO QUALIFIES

Beneficiaries who lost employer/union/COBRA coverage. Can enroll in MA, MAPD, or PDP.

COMMON MISTAKES

Calculating from the call date instead of the coverage end date. Not recognizing COBRA expiration as a qualifying loss. Not checking whether Part B was also delayed (ICEP may apply simultaneously). Not checking for remaining VA or TRICARE drug coverage (CDC may also apply).

TALK TRACK

"Because your employer coverage just ended, you qualify for a Special Enrollment Period right now. That gives you a window from when that coverage ended to get into a new Medicare plan. When exactly did your coverage terminate?"

REAL-WORLD SCENARIO

Mrs. Martinez just retired last month and her employer coverage ended March 31st. She has had Part A since turning 65 but delayed Part B — which is now activating May 1st. This is both LEC and ICEP. You document both codes and enroll her in the MAPD plan that best fits.

Delayed Part B forms to know: CMS-L564 (Request for Employment Information) is completed by the employer to verify the beneficiary had employer coverage. CMS-40B (Application for Enrollment in Medicare Part B) is the actual Part B enrollment form submitted to Social Security. Reference these form numbers on the call to build credibility.

OSD

Cost Plan Disenrollment SEP

The beneficiary dropped a Medicare Cost Plan that included drug coverage and needs a standalone PDP.

WHAT TRIGGERS IT

Dropping a Medicare Cost Plan that included Part D drug coverage and returning to Original Medicare. Cost Plans are a distinct plan type operating in limited markets, primarily in the Midwest (Minnesota, Iowa, and surrounding areas).

ENROLLMENT WINDOW

Two full calendar months after the month they drop the Cost Plan.

WHO QUALIFIES

Beneficiaries who dropped a Cost Plan with drug coverage. Must enroll in PDP only — OSD does not apply to MA/MAPD enrollment.

COMMON MISTAKES

Not knowing what a Cost Plan is (they are rare and market-specific). Trying to use OSD for MA enrollment. Applying OSD when the dropped Cost Plan did not include drug coverage.

TALK TRACK

“Since you left your Cost Plan, you have a two-month window to get into a drug plan so your prescriptions stay covered. Let me find the best option for your medications.”

REAL-WORLD SCENARIO

Mr. Hansen from Minnesota just dropped his HealthPartners Cost Plan last month. The plan included Part D drug coverage. He needs a standalone PDP now. OSD gives him two months.

12G

12-Month Trial Right (Medigap)

The beneficiary dropped a Medigap plan to join their very first MA/MAPD plan and now wants to go back. They have 12 months to return to Original Medicare plus Medigap plus PDP.

WHAT TRIGGERS IT

The beneficiary specifically dropped a Medicare Supplement (Medigap) policy to enroll in their first-ever MA/MAPD plan. Now they want to return.

ENROLLMENT WINDOW

12 months from the MA plan effective date.

WHO QUALIFIES

First-time MA enrollees who dropped a Medigap plan to join MA. Returns them to Original Medicare plus Medigap (with guaranteed issue rights varying by state) plus standalone PDP.

COMMON MISTAKES

Confusing 12G with 12J. Key difference: 12G requires a prior Medigap plan that was dropped. 12J does not require Medigap history. Trying to use 12G for someone with prior MA history — it must be their first time in MA.

TALK TRACK

“Since you dropped your Medigap plan to try Medicare Advantage for the first time and it has been less than 12 months, you have the right to go back to Original Medicare with your supplement. Let me help you get that set up.”

REAL-WORLD SCENARIO

Mrs. Sullivan had a Medigap Plan G for years. During last AEP, she switched to an MAPD plan for the first time (effective January 1st). It is now June, and she hates it. She dropped her Medigap to join MA, this is her first MA plan, and it has been less than 12 months. 12G applies.

Practical context: If you sold MAPD plans during the most recent AEP and any of those clients had Medigap before, those clients have a 12G trial right through the end of the current calendar year. Proactively check in — if they are happy, you solidify the relationship. If unhappy, you help them exercise 12G before the window closes. The 12-month clock starts from the MA plan effective date (usually January 1st), not from the application signing date.

12J Age-65 Trial Right

The beneficiary enrolled in MA/MAPD for the first time when they turned 65 and wants to go back to Original Medicare plus PDP within 12 months.

WHAT TRIGGERS IT

First-time MA/MAPD enrollment upon turning 65. The beneficiary wants to leave MA within the first year.

ENROLLMENT WINDOW

12 months from the initial MA plan effective date.

WHO QUALIFIES

First-time age-65 MA enrollees. Must also enroll in a standalone PDP — PDP enrollment is mandatory under 12J. Returns to Original Medicare plus PDP.

COMMON MISTAKES

Not enrolling in a PDP — it is mandatory, not optional, under 12J. Confusing with 12G — 12J does not require a prior Medigap plan. Applying 12J to someone who was not a first-time age-65 enrollee.

TALK TRACK

“Since you enrolled in Medicare Advantage for the first time when you turned 65 and it has been less than a year, you have the option to return to Original Medicare. We will also need to set you up with a drug plan. Let me walk you through the options.”

REAL-WORLD SCENARIO

Mr. Reynolds turned 65 in January and enrolled in an MAPD plan effective January 1st. It is now September. He feels Original Medicare would give him more flexibility. He has no Medigap history (so 12G does not apply), but 12J does — first-time age-65 MA enrollment within 12 months.

CDC

Creditable Drug Coverage SEP

The beneficiary is in an MAPD plan but also has other creditable drug coverage (VA, TRICARE, employer retiree drug) — they can move to an MA-only plan to drop the duplicate drug coverage.

WHAT TRIGGERS IT

Beneficiary enrolled in MAPD (or PDP) who has other active creditable drug coverage. They are paying for drug coverage they do not need because VA, TRICARE, or employer retiree coverage already handles it.

ENROLLMENT WINDOW

Anytime. No window restriction.

WHO QUALIFIES

Beneficiaries in MAPD or PDP with other active creditable drug coverage. Must move OUT of MAPD/PDP and INTO an MA-only plan — the Part D component must be dropped.

COMMON MISTAKES

Trying to use CDC to switch between MAPD plans — it is not a switching tool. The destination must be MA-only. Not verifying that the other creditable drug coverage is still active.

TALK TRACK

“Since you already have drug coverage through [VA/TRICARE/employer], you are actually paying for duplicate drug coverage in your current plan. We can move you to a Medicare Advantage plan without the drug component, which could save you money. And we can do this at any time.”

REAL-WORLD SCENARIO

Mr. Cooper is in an MAPD plan but also has TRICARE for Life, which includes drug coverage. He is effectively paying for Part D twice. CDC lets him move to an MA-only plan, drop the duplicate Part D, and potentially lower his costs. Available anytime.

DIF

Government Enrollment SEP

Medicare automatically enrolled the beneficiary in a plan without their choice. They have three months to switch to whatever they actually want.

WHAT TRIGGERS IT

Government auto-enrollment. Medicare placed the beneficiary into a plan without their active selection. In MARx, look for an "X" indicator next to the plan code.

ENROLLMENT WINDOW

Three months from the auto-enrollment effective date.

WHO QUALIFIES

Beneficiaries who were automatically enrolled by the government. Can switch to any MA, MAPD, or PDP.

COMMON MISTAKES

Not checking for the "X" indicator in MARx. Not recognizing the trigger — the beneficiary may not know how they got into their plan. Common with low-income beneficiaries automatically assigned to benchmark LIS drug plans.

TALK TRACK

"It looks like Medicare placed you in this plan automatically — you did not choose it yourself. That means you have a three-month window right now to switch to whatever plan actually fits your needs. Let me help you find the right one."

REAL-WORLD SCENARIO

Mrs. Nelson calls confused. She does not recognize her current drug plan. MARx shows an "X" next to the plan code — government auto-enrollment. She is a low-income beneficiary automatically assigned to a benchmark LIS plan. DIF applies — three months to switch.

ACC

Accessible Format SEP

The beneficiary requested plan materials in an accessible format (large print, Braille, audio) and did not receive them in time to make an enrollment decision during their available window.

WHAT TRIGGERS IT

The beneficiary requested accessible format materials and either did not receive them or received them too late to make an informed enrollment decision within an election period they already had open.

ENROLLMENT WINDOW

Equal to the time lost waiting for accessible materials — essentially extends the enrollment period the beneficiary missed due to the delay.

WHO QUALIFIES

Beneficiaries who genuinely need accessible format materials and were unable to make a timely enrollment decision because of the delay in receiving them.

COMMON MISTAKES

This is a heavily watched SEP. Carrier benchmarks are approximately 3% of enrollments. Do NOT use ACC as a convenience code. The beneficiary must have actually requested accessible materials and been unable to enroll during their window because of the delay.

TALK TRACK

“It sounds like you needed those materials in a different format and did not get them in time to make your decision. That actually qualifies you for additional time to make your enrollment choice. Let me help you with that now.”

REAL-WORLD SCENARIO

Mrs. Williams is legally blind and requested large-print plan comparison materials during AEP. The materials arrived in mid-December — after AEP closed on December 7th. She was unable to compare plans. ACC gives her additional time. You verify she actually requested the materials and confirm the dates.

Comparison: 12G vs 12J

	12G	12J
The one question	Did they drop a Medigap plan to join MA?	Did they join MA for the first time at 65?
Prior Medigap required?	Yes — must have dropped Medigap to join MA	No — no Medigap history needed
PDP required?	Yes	Yes — mandatory
Window	12 months from MA effective date	12 months from MA effective date
Returns to	Original Medicare + Medigap (guaranteed issue) + PDP	Original Medicare + PDP

Chapter 8: Star Ratings

CMS rates every Medicare plan on a star system. Two SEPs are tied directly to those ratings.

5ST

5-Star SEP

A 5-star rated plan is available in the beneficiary's area, and they can enroll in it outside of AEP.

WHAT TRIGGERS IT

The existence of a CMS 5-star rated MA, MAPD, or PDP in the beneficiary's service area. This is the only SEP that allows year-round enrollment without a qualifying life event — the "event" is simply that a top-rated plan exists nearby.

ENROLLMENT WINDOW

December 8th through November 30th of the following year. Once per calendar year.

WHO QUALIFIES

Any beneficiary with a 5-star plan available in their ZIP code. One use per year.

COMMON MISTAKES

Not verifying that the plan still holds 5 stars for the current plan year — designations change annually. Using 5ST twice in the same year. Offering 5ST without confirming a 5-star plan actually exists in the beneficiary's specific ZIP code.

TALK TRACK

"There is a plan in your area that earned the highest quality rating from Medicare — five out of five stars. Because of that rating, you can actually enroll in it right now, outside of the normal enrollment windows."

REAL-WORLD SCENARIO

Mrs. Campbell calls about her current plan. You check carrier portals and discover a 5-star rated MAPD plan in her county. Even though it is July and no other SEP applies, 5ST lets her switch.

LPI

Low-Performing Plan SEP

The beneficiary is stuck in a plan that has been rated 2.5 stars or lower for three straight years. They can leave at any time.

WHAT TRIGGERS IT

The beneficiary's current plan has a CMS star rating of 2.5 or lower for three consecutive contract years. CMS designates these as low-performing plans.

ENROLLMENT WINDOW

Anytime while enrolled in a low-performing plan. No window restriction.

WHO QUALIFIES

Beneficiaries currently in a low-performing plan. Must enroll into a plan rated 3 stars or higher.

COMMON MISTAKES

Not checking the plan's current star rating. Enrolling the beneficiary into another plan rated below 3 stars — the destination must be 3 stars or higher.

TALK TRACK

"Your current plan has been rated below average by Medicare for three years in a row. You do not have to stay in it — you can switch to a higher-rated plan at any time. Let me show you what is available."

REAL-WORLD SCENARIO

Mr. Webb calls referencing a CMS letter about his plan being "low-performing." You verify in MARx that his plan is rated 2.0 stars for three consecutive years. LPI applies — he can switch at any time to a plan rated 3 stars or higher.

Chapter 9: Disaster

One SEP in this category, and it is the most misunderstood code in the entire system.

DST

Disaster SEP

A FEMA-declared disaster prevented the beneficiary from making an enrollment during a window they already had open. The disaster extends that missed window — it does NOT create a new one.

WHAT TRIGGERS IT

A FEMA-declared disaster or CMS-designated public health emergency affected the beneficiary's area during a time when they had an active enrollment window (AEP, IEP, OEP, or another SEP) and the disaster prevented them from using it.

ENROLLMENT WINDOW

Duration of the emergency plus two full months after the declared end date.

WHO QUALIFIES

Beneficiaries who (1) were in an area affected by a FEMA declaration, (2) had an active enrollment window during the disaster period, (3) were prevented from enrolling because of the disaster, and (4) did NOT use any other SEP since the disaster. All four conditions must be met.

COMMON MISTAKES

The biggest one: treating DST as a standalone SEP. A disaster alone does NOT create a new enrollment window. The beneficiary must have had an existing window that was disrupted. If a hurricane hit in August but the beneficiary did not have any active SEP during August, DST does not apply. Also, if the beneficiary used ANY other SEP after the disaster, DST is no longer available. Do NOT proactively market or suggest this SEP.

TALK TRACK

"It sounds like the disaster in your area may have prevented you from enrolling during your window. If that is the case, you may have additional time. Let me check the FEMA declarations for your area and see if we can help."

REAL-WORLD SCENARIO

Mr. Sanchez calls in April. A hurricane hit his area in October, during AEP. He was displaced and could not complete his AEP enrollment. He has not used any other SEP since. The FEMA declaration for his county is still active. DST applies — the disaster extended his missed AEP window.

DST requires two enrollment reasons on the application: (1) the election period the beneficiary missed (AEP, IEP, OEP, etc.), and (2) "Affected by an emergency or major disaster (as declared by FEMA or a government entity)."

DST Pre-Submission Checklist: Is there an active FEMA declaration for the beneficiary's county? Do the dates overlap with an enrollment period? Which enrollment period did the beneficiary miss? Was the beneficiary actually prevented from enrolling? Has the beneficiary used any other SEP since? Did the beneficiary raise the disaster situation (you should never bring it up)?

COMPLIANCE WARNING: DST IS THE MOST WATCHED SEP

Carrier compliance teams flag DST usage harder than any other code. Benchmark is approximately 15%. Three non-negotiable rules:

1. Never advertise or market DST. The beneficiary must raise the situation first.
2. The beneficiary must have missed another enrollment period. DST extends a missed window — it does not create a new one.
3. Verify the FEMA declaration. Confirm the county is covered and dates overlap with the missed enrollment period.

Chapter 10: Election Periods

Not technically SEPs — standing enrollment periods on a fixed annual schedule. You must know them cold because they interact with every SEP.

AEP

Annual Election Period

October 15th through December 7th. Everyone can change plans. No SEP needed. Coverage effective January 1st.

WHAT TRIGGERS IT

The calendar. October 15th rolls around and the door opens for every Medicare beneficiary.

ENROLLMENT WINDOW

October 15 through December 7 each year. Coverage effective January 1st of the following year. The last enrollment submitted wins.

WHO QUALIFIES

All Medicare beneficiaries. No restrictions.

COMMON MISTAKES

Confusing AEP with OEP — AEP is October through December for everyone; OEP is January through March for existing MA members only. Not realizing that submitting multiple plans during AEP is valid — only the last one takes effect.

TALK TRACK

“We are in open enrollment right now, which means we can look at all the plans available to you and get you enrolled today. Whatever we choose will take effect January 1st.”

REAL-WORLD SCENARIO

Anyone calling between October 15th and December 7th. No SEP discovery needed. Confirm their Medicare status, compare plans, enroll. The simplest calls of the year.

OEP

MA Open Enrollment Period

January 1st through March 31st. Existing MA members get one change. Effective the first of the following month.

WHAT TRIGGERS IT

The calendar, plus the beneficiary must already be enrolled in an MA or MAPD plan.

ENROLLMENT WINDOW

January 1 through March 31. One change only. Effective the 1st of the following month.

WHO QUALIFIES

Beneficiaries currently enrolled in an MA or MAPD plan. NOT available to PDP-only or Original Medicare beneficiaries.

COMMON MISTAKES

Treating OEP as a free-for-all like AEP — it is one change only, for existing MA members only. Not checking MARx to see if OEP has already been used. Using OEP for a dual-eligible beneficiary who has INT or DEP available — those monthly SEPs are better options.

TALK TRACK

“Since you are already in a Medicare Advantage plan, you have the option to make one change during this open enrollment window. That change would take effect the first of next month.”

REAL-WORLD SCENARIO

Mr. Perez calls in February. He enrolled in an MAPD during AEP but wants to switch to a different one. He is not dual-eligible, not new to Medicare, and has no other qualifying event. But he is in an MA plan during OEP — he can make one change.

The Confusion Points

These are the code pairs that agents mix up most. Master these distinctions and you will avoid the most common enrollment errors.

IEP vs ICEP

“Do Part A and Part B have the same effective date?”

- Same date → IEP. 7 months around birthday.
- Different dates → ICEP. 5 months around Part B start.
- Disability turning 65 → IEP2. Fresh 7-month window.
- Already in effectuated MA → OEP-N. Not IEP.
- Notified retroactively → RET.

INT vs DEP

“Does the beneficiary have FULL Medicaid?”

- Full Medicaid (FBDE, QMB+, SLMB+) → INT for D-SNP. Any month.
- Any Medicaid or LIS (including QMB-only) → DEP for PDP. Any month.
- QMB-only → DEP only, NOT INT.
- Income shortcut: under \$1,903/month single → likely QMB or higher.

EOC vs MYT

“Did the carrier choose to leave, or did Medicare terminate the contract?”

- Carrier letter (ANOC) → EOC. Dec 8 through end of Feb.
- CMS letter → MYT. 2 months before + 1 month after contract end.

MOV vs LEC

“Did they change their physical address, or did they lose health coverage?”

- Moved → MOV. PO Box does not count.
- Lost employer/COBRA → LEC. COBRA expiration counts.
- Can overlap — retired and moved = both LEC and MOV.

12G vs 12J

“Did they drop a Medigap plan to join MA?”

- Had Medigap, dropped it for first MA → 12G. Returns to OM + Medigap + PDP.
- First MA at 65, no Medigap history → 12J. Returns to OM + PDP (mandatory).

OEP-I vs LTC

“Do they need MA/MAPD, or do they need a standalone PDP?”

- MA/MAPD in nursing home → OEP-I.
- PDP in nursing home → LTC.
- Same facilities. Same windows. Different plan types.

LCC vs LEC

“Was it employer/union/COBRA coverage, or was it other creditable coverage?”

- Employer/union/COBRA → LEC. Retirement and COBRA expiration count.
- VA/TRICARE/ACA/other → LCC. Must be involuntary.

The Enrollment Calendar

Know where you are in the calendar and you will know what tools are available.

January

ACTIVE PERIODS

MA OEP begins (Jan 1). EOC window open (through end of Feb). All SEPs available.

WHAT TO THINK ABOUT

Dual-eligible auto-assigned plans (DIF). New MA enrollees unhappy with Jan 1 plan (OEP-N). Medicaid redetermination changes (MCD/NLS).

February

ACTIVE PERIODS

OEP continues. EOC window closing at end of month.

WHAT TO THINK ABOUT

Deadline pressure for EOC. OEP-N beneficiaries still have time. LIS changes from January redeterminations (NLS).

March

ACTIVE PERIODS

OEP final month — closes March 31st.

WHAT TO THINK ABOUT

Urgency messaging for OEP. After March 31st, only valid SEPs allow enrollment. Ensure dual-eligible beneficiaries know they have INT/DEP.

April

ACTIVE PERIODS

SEP Season begins. No general enrollment period open.

WHAT TO THINK ABOUT

Every call requires discovery. Train on the six signal cards: address mismatch (MOV), Medicaid/LIS (INT/DEP), chronic condition (CSN), plan ending (EOC/MYT), lost coverage (LEC/LCC), new to Medicare (IEP/ICEP).

May – September

ACTIVE PERIODS

Deep SEP Season. 5-Star SEP available (through Nov 30).

WHAT TO THINK ABOUT

Discovery mindset is everything. Listen for triggers. Ask about recent changes. MCD and NLS available past September 30th.

October 1–14

ACTIVE PERIODS

Still SEP Season — AEP starts Oct 15.

WHAT TO THINK ABOUT

Pre-AEP energy. If they have a qualifying event, help now. If not, AEP is days away.

Oct 15 – Dec 7 (AEP)

ACTIVE PERIODS

Annual Election Period. Open to all.

WHAT TO THINK ABOUT

Highest volume. Still identify Medicaid status for plan selection. IEP/ICEP still active. Last enrollment submitted wins.

December 8–31

ACTIVE PERIODS

AEP closed (Dec 7). EOC opens Dec 8. 5-Star SEP active.

WHAT TO THINK ABOUT

Plan non-renewals (EOC). 5-star enrollments. IEP/ICEP for December birthdays. Preparing for OEP starting Jan 1.

Quick Decision Tree

A beneficiary calls. You do not know what SEP they have. Here is the path.

Step 1: What Season Is It?

- Oct 15 – Dec 7? AEP is open. Enroll without an SEP. Still identify Medicaid, chronic conditions, etc.
- Jan 1 – Mar 31? OEP is open for existing MA members. Check: are they already in an MA plan? If yes, one change.
- Apr 1 – Oct 14? SEP Season. You must find a qualifying event or you cannot help them.

Step 2: Are They New to Medicare?

- Part B started recently with same A/B dates → IEP
- Part B started recently with different A/B dates → ICEP
- Disability beneficiary turning 65 → IEP2
- Already in MA plan that effectuated and wants to change → OEP-N
- Notified of Medicare after it started → RET

Step 3: Do They Have Medicaid or Extra Help?

- Full Medicaid → INT (D-SNP, any month)
- Any Medicaid or LIS → DEP (PDP, any month)
- Medicaid or LIS recently changed → MCD or NLS (3-month window)

Step 4: Did Something Change?

- Moved → MOV. Confirm new address, move date. PO Box does not count.
- Lost employer/union/COBRA coverage → LEC. Get the date.
- Lost VA/TRICARE/ACA/other creditable coverage → LCC. Must be involuntary.
- Plan ending (carrier left) → EOC. Plan ending (CMS terminated) → MYT.
- Released from incarceration → INC. Two months.
- Returned from living abroad → RUS. Two months.
- Became a US citizen → LAW. Month of change + 2 months.
- Medicare auto-enrolled them → DIF. Three months.

Step 5: Chronic Condition or Special Situation?

- Chronic condition + C-SNP available → CSN. Check carrier portals.
- In a nursing home/SNF → OEP-I (MA/MAPD) or LTC (PDP)
- In a State Pharmaceutical Assistance Program → PAP
- Left a PACE program → PAC. Two months.
- Lost SNP eligibility → SNP. Act fast.
- Has other drug coverage + in MAPD → CDC. Switch to MA-only anytime.

Step 6: Star Ratings and Trial Rights

- 5-star plan available → 5ST. Dec 8 through Nov 30.
- Plan rated 2.5 stars or below for 3 years → LPI. Anytime.
- Dropped Medigap for first MA within 12 months → 12G
- First MA at 65 within 12 months → 12J
- Lost Part B and lost MA → INV. PDP only.
- Carrier under state receivership → REC
- Dropped a Cost Plan with drug coverage → OSD. PDP only.

Step 7: Disaster Extension

- FEMA disaster prevented enrollment during an existing window → DST
- Verify: (1) active FEMA declaration, (2) they had an open window, (3) no other SEP used since, (4) they raised it first.

Step 8: No SEP Found

- Be honest: "Based on what you have shared, I do not see a qualifying event right now."
- Tell them when the next window opens (AEP/OEP).
- Let them know if anything changes (move, lose coverage, Medicaid change), call right away.
- Do not force it. Do not fabricate an SEP.

Compliance

The Election Period Verification Rule

Before every SEP enrollment, you must verify the election period with the beneficiary. This is not optional — it is a CMS requirement. You must ask the questions, discuss the qualifying event, and verify the election period being used with the beneficiary before submitting the enrollment. Election Period Verification is not a checkbox you click at the end. It is a conversation you have before you start the application.

"If you cannot explain to the beneficiary which SEP they are using and why they qualify for it, you should not be submitting that enrollment."

The 5 Most Watched SEPs

Carriers track SEP usage rates against industry benchmarks. If your rates exceed these thresholds, expect a compliance review.

CODE	NAME	BENCHMARK	WHY IT IS WATCHED
DST	Disaster SEP	~15%	Agents treat it as a free enrollment ticket during hurricane season. It is an extension of a missed window, not a new one.
MOV	Moving SEP	~10%	Agents accept PO Box changes or unverified address changes as valid moves. Address must be verified with Social Security.
LCC	Loss of Creditable Coverage	~4%	Agents use LCC for voluntary coverage lapses (missed premiums). Must be involuntary loss only.
ACC	Accessible Format	~3%	Agents use ACC as a convenience code when no other SEP applies. Must be a genuine accessibility need.
PAP	SPAP	~5%	Agents enroll beneficiaries who are not actually in a qualifying state pharmaceutical assistance program.

What Happens When You Submit the Wrong SEP

- Enrollment rejection. The beneficiary is left without the plan they thought they were getting.
- Retroactive disenrollment. CMS can retroactively disenroll the beneficiary months later.
- Carrier compliance review. Your enrollment patterns are flagged with mandatory retraining.
- Suspension of enrollment privileges. The carrier suspends your ability to submit enrollments.
- Contract termination. Repeated violations lead to carrier contract termination and potential CMS sanction.

The “Ask Before You Submit” Protocol

Before every SEP enrollment, run through these five questions. If you cannot answer all five with confidence, stop and verify.

- **1. What is the qualifying event?**

Can you name the specific life event that triggered this SEP?

- **2. When did it happen?**

Do you have the specific date? Is the window still open?

- **3. Does the beneficiary understand?**

Can the beneficiary explain in their own words why they qualify? "Okay" is not verification.

- **4. Is this the right SEP code?**

Are you using the most appropriate code per the CMS hierarchy?

- **5. Is the plan type valid?**

Does this SEP code allow enrollment in the plan type you are submitting?

Red Flags That Get You Flagged

- High DST volume without a matching FEMA declaration.
- MOV enrollments without address verification in MARx.
- ACC usage above 3% of total enrollments.
- Multiple SEP codes on the same beneficiary in a short period.
- SEP enrollments clustered at the end of enrollment windows.
- LCC for beneficiaries who simply stopped paying premiums.

The DO NOT List

- DO NOT enroll Medicaid beneficiaries in give-back plans. Medicaid pays their Part B premium — the give-back goes nowhere.
- DO NOT advertise, market, or proactively contact beneficiaries about the Disaster SEP.
- DO NOT assume or infer an SEP. The qualifying event must be confirmed by the beneficiary and verified in the system.
- DO NOT use ACC for a beneficiary who does not actually need accessible format materials.
- DO NOT submit an enrollment without verifying the election period with the beneficiary first.
- DO NOT use DST as a standalone SEP — it is an extension of a missed window, not a new enrollment period.
- DO NOT use LCC when the beneficiary voluntarily dropped their coverage or missed premium payments.
- DO NOT submit an INT enrollment without confirming the D-SNP is an integrated plan (FIDE, HIDE, or AIP) and that the Medicaid MCO aligns with the carrier.

When in Doubt

- Check the SEP Check tool in the Certainty System at /sep-check.
- Check carrier portals (Humana Vantage/Mentor DMS-024, UHC Election Period Booklet, etc.).
- Contact your upline for edge cases.
- Contact carrier compliance before submitting if the situation is genuinely ambiguous.

The single best habit you can develop: if you are not 100% sure the SEP is valid, pause and verify. The five minutes it takes to confirm will save you weeks of dealing with a compliance review.

All 37 Codes Reference Table

CODE	FULL NAME	TRIGGER	WINDOW	KEY DETAIL
IEP	Initial Enrollment Period	Turning 65 / first Medicare eligibility	7 months (3 before + birthday month + 3 after)	Part A and Part B must share same effective date. NOT valid for MA-only plans.
IEP2	IEP2 (Fresh Window at 65)	Disability beneficiary turning 65	7 months around 65th birthday	Uses MRD on application. Watch for turning-65 DOB. MA-only prohibited.
ICEP	Initial Coverage Election Period	Delayed Part B, now activating	5 months (3 before + Part B month + 1 after)	Part A/B have different effective dates. NOT valid for PDP. Recently extended.
OEP-N	New Enrollee OEP	First MA/MAPD effectuated, wants to change	Month of effectuation + 2 months	One change only. Not for PDP-only or Medigap-only beneficiaries.
RET	Retroactive Entitlement	Notified of A/B after coverage already started	Month of notice + 2 months	Clock starts from notification date, not coverage date.
INT	Integrated Care SEP	Full Medicaid, enrolling in D-SNP	Any month, repeatable	Requires FBDE, QMB+, SLMB+, or Full Medicaid. D-SNP enrollment only.
DEP	Dual/LIS Monthly SEP	Any Medicaid or LIS, PDP change	Any month, repeatable	Any Medicaid level or LIS qualifies. PDP enrollment only.
MCD	Medicaid Change SEP	Gained/lost/changed Medicaid level	3 months from change	Available all year including after September 30th.
NLS	Extra Help Change SEP	Gained/lost/changed LIS level	3 months from change	Available all year including after September 30th. Watch for SS letters.
MOV	Change of Residence SEP	Moved to new ZIP or county	Month before (if notified) + month of + 2 after	NOT valid for PO Box change. Physical address must change.
INC	Post-Incarceration SEP	Released from correctional facility	2 months after release	Medicare suspended, not terminated, during incarceration.
RUS	Return to US SEP	Returned from living permanently abroad	2 months after return	Short trips/vacations do not qualify.
LAW	Lawful Presence SEP	Became US citizen or gained qualifying status	Month of status change + 2 months	Citizenship does not auto-start Medicare. Confirm Part A/B.
CSN	C-SNP Eligibility SEP	Qualifying chronic condition + C-SNP available	Once per calendar year	Must enroll INTO a C-SNP. Provider attestation required within 2 months. Not valid C-SNP to C-SNP same condition.
PAP	SPAP SEP	Enrolled in state pharmacy assistance program	1x/year while enrolled; 2 months after loss	Common programs: NY EPIC, NJ PAAD, PA PACE/PACENET, WI SeniorRx.
PAC	PACE Disenrollment SEP	Left PACE program	2 months after disenrollment	NEVER suggest or initiate PACE disenrollment.
SNP	SNP Loss SEP	Lost SNP eligibility / condition not verified	Up to 3 months after grace period ends	Often due to provider failing to verify within 2 months.

OEP-I	Institutionalized OEP	In nursing home/SNF/LTC facility	Unlimited while in facility + 2 months after	MA/MAPD only. Assisted living does NOT qualify.
LTC	LTC SEP	In nursing home/SNF/LTC facility	Unlimited while in facility + 2 months after	PDP only. Same facilities as OEP-I but different plan type.
LCC	Loss of Creditable Coverage	Lost VA, TRICARE, ACA, other creditable coverage	2 months from loss or notification (whichever later)	NOT valid if beneficiary missed premium payments.
INV	Involuntary Loss SEP	Lost MAPD because Part B terminated	Notice + grace period + 2 months after	PDP only. Cannot re-enter MA until Part B restored.
REC	Receivership SEP	Plan carrier under state financial receivership	Until state action ends or member switches	Rare. Verify with compliance team. Not same as EOC.
EOC	Plan Non-Renewal SEP	Carrier ended plan in service area	December 8 – end of February	Carrier-initiated market exit. Beneficiary received ANOC letter.
MYT	Medicare Contract Termination	CMS terminated contract with carrier	2 months before + 1 month after end	CMS enforcement action. Formal CMS letter to beneficiary.
LEC	Loss of Employer Coverage SEP	Lost employer/union/COBRA coverage	Month of loss + 2 months after	COBRA expiration counts. May coincide with ICEP. Know CMS-L564 and CMS-40B forms.
OSD	Cost Plan Disenrollment SEP	Dropped Cost Plan with drug coverage	2 full months after drop	PDP only. Cost Plans primarily in Midwest markets.
12G	12-Month Trial Right (Medigap)	Dropped Medigap for first MA, wants to return	12 months from MA effective date	Must have dropped Medigap to join first-ever MA plan.
12J	Age-65 Trial Right	First MA enrollment at 65, wants to leave	12 months from MA effective date	PDP enrollment mandatory. No Medigap history required.
CDC	Creditable Drug Coverage SEP	In MAPD but has other drug coverage (VA/TRICARE)	Anytime	Must move OUT of MAPD into MA-only plan.
DIF	Government Enrollment SEP	Auto-enrolled into plan by Medicare	3 months from effective date	Look for "X" indicator in MARx. Common with LIS auto-assignments.
5ST	5-Star SEP	5-star plan available in area	December 8 – November 30 (once per year)	Verify plan still holds 5 stars for current year.
LPI	Low-Performing Plan SEP	Plan rated 2.5 stars or below for 3 years	Anytime while enrolled	Must enroll into plan rated 3+ stars.
DST	Disaster SEP	FEMA disaster prevented enrollment	Duration of emergency + 2 months after end	Extension only — not standalone. Must have had active window. Use pre-submission checklist.
ACC	Accessible Format SEP	Requested accessible materials, not received in time	Equal to time lost	Only for genuine accessibility needs. Benchmark ~3%. Heavily audited.
AEP	Annual Election Period	Calendar (Oct 15 – Dec 7)	Coverage effective January 1. Last plan wins.	Open to all. No SEP required. Multiple submissions OK.
OEP	MA Open Enrollment Period	Calendar (Jan 1 – Mar 31) for existing MA members	One change. Effective 1st of following month.	MA members only. One change per year. MARx shows if used.
GEP	General Enrollment Period	Missed IEP, signing up for Part B	Jan 1 – Mar 31. Coverage effective 1st of month after sign-up.	For beneficiaries who missed their IEP. May incur late enrollment penalty.

This document contains all 37 SEP codes with complete details. For the most current FEMA declarations and real-time SEP verification, use the SEP Check tool at </sep-check>.